

HALF-BUILT AMBITION
An AIIMS building under construction in Deoghar, Jharkhand

India Institute of Medical Sciences (AIIMS) in Delhi—operates well below its sanctioned strength. Of the total sanctioned faculty of 4,209 in 19 AIIMS, 2,317 or 55 per cent positions are vacant, as the Union government revealed in the Lok Sabha on December 3, 2021. The Delhi AIIMS has 97 vacancies in the faculty as against the sanctioned strength of 305.

Professor K. Srinath Reddy, president of the Public Health Foundation of India in Delhi, has a way around this conundrum. "Faculty numbers will expand when specialists serving in district hospitals are recognised and designated as faculty," he says. "Shared classrooms through IT connectivity, adjunct and visiting faculty as well as online education are methods to overcome shortages till we can produce more permanent teaching staff."

Australia-based Dr Shailja Chaturvedi, a member of the executive council of the Global Association of Physicians of Indian Origin, suggests that the government call upon emigrant doctors for short- or long-term engagement. "This could help in bridging the existing gap without compromising quality," she says.

PRIVATE PROFIT

Many observers attribute the decline in the Indian medical education system to the surge in private medical colleges, especially in the past three decades. The number of private medical colleges in the country has gone up to 269 from 47 in 1990, a growth of 82 per cent. However, with the exception of legacy institutions such as the Christian Medical College, Vellore, St. John's Medical College in Bengaluru and Kasturba Medical College in Manipal, most of the newer private medical colleges were perceived to be substandard and driven solely by the profit motive. Marred by shortages of faculty, infrastructure facilities and patient load, all these institutes have managed to do is produce poor-quality medical graduates.

Industry insiders claim that setting up a private medical college requires an investment of up to Rs 500 crore. The exorbitant cost of land—the campus of a private college must be spread over an area of 20 acres—also forces founders to set up such colleges away from big cities. This leads to two kinds of problems—low patient footfall and the dearth of permanent faculty. In an attempt to encourage the private sector, the Union government has over a period of time relaxed norms for land, faculty, staff, bed strength and other infrastructure for setting up a medical college.

1.3 MILLION ALLOPATHIC DOCTORS IN INDIA

Most private medical colleges claim they charge much less than what is required to provide quality medical education and do not have the leeway that government colleges have to subsidise it further. AIIMS pegs the cost of producing one MBBS at Rs 1.7 crore, but the student pays just a fraction of this. In a bid to regulate the fee structure in private medical colleges, the NMC has now mandated that the fees for 50 per cent of the seats in private medical colleges and deemed universities be kept at par with government medical colleges in the same state or Union territory.

However, as critics say, this will lead to private medical colleges overcharging for the other 50 per cent seats to compensate for the revenue loss, and render them further out of reach for more students. "The unregulated 50 per cent seats will now be legally sold to the highest bidder," says Dr R.V. Asokan, honorary secretary-general of the Indian Medical Association (IMA), a voluntary body of doctors in India. Dr Anup Karan, additional professor at the Indian Institute of Public Health, Delhi (IIPHD), also believes that capping fees will only breed corruption and inefficiency, and offers a pragmatic solution instead. "The government must announce a common but liberal scholarship system for the poor and other less advantaged population groups. If health insurance can be designed for the poor, why not medical education?" he asks.

Private colleges have also shown tremendous resourcefulness in circumventing government attempts to